17.10 Congenital Heart Disease (VSD and Tetralogy of Fallot)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Mother says, "He turns blue when he cries." **Objective** • Cyanosis; SpO_2 78% • Hypercyanotic ("tet") spells • Finger clubbing	**ACTUAL** Impaired gas exchange related to right-to-left shunting as evidenced by cyanosis and low SpO_2.	• Spells become fewer and shorter.	• During a spell: knee–chest position, oxygen and medication as prescribed. • Keep him calm. • Prevent dehydration. • Prepare for surgical repair.	• Met when spells are controlled.
Subjective • Mother says, "He gets tired and sweaty when feeding." **Objective** • Poor weight gain • Sweats during feeds	**ACTUAL** Ineffective infant feeding dynamics related to fatigue during feeding as evidenced by diaphoresis and poor weight gain.	• Steady weight gain.	• Offer small, frequent feeds. • Give high-energy feeds as prescribed. • Let him rest before feeds. • Weigh regularly.	• Achieved as he gains weight.
Subjective *No specific complaint* **Objective** • Heart defect • Frequent chest infections	**RISK** Risk for infection as evidenced by risk factors: congenital heart defect (endocarditis, respiratory infection).	• No infection.	• Make sure immunizations are complete. • Teach oral hygiene. • Ask the parents to report fever.	• Outcome reached if no infection develops.

*References for this care plan: [16], [35], [38]. Full entries are listed in the References section of the CPMN book.